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Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
PRESIDENTIAL EBOLA TASK FORCE 17
MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°146 /MVE-BDBV/07/10/2026

Report date: October 7, 2026 Publication date: October 8, 2026

HIGHLIGHTS (last 24 hours)

ÿ **Fifty-nine (59) new confirmed cases of Ebola Virus Disease (EVD), including 26 deaths**
Community transmission cases were reported. These cases originated from the provinces of Ituri (29 cases) and North Kivu (26 case
and Haut-Uele (4). In addition, 11 deaths were recorded in the CTEs

ÿ **A total of 393 new samples were analyzed; 59 of these samples tested positive for the virus**
Bundibugyo, or 15.0% positivity.

ACCUMULATIVE CAS	ACCUMULATIVE DEATH CONFIRMED	LETHALITY	PATIENTS IN ISOLATION/CTE	ACCUMULATIVE HEALED	TRACKING RATES CONTACTS (Of the Day)
8 787	4 242	48,3 %	980	2 292	80,0%

Affected provinces and affected health zones

7 Provinces affected

Uele, Ituri, North Kivu, South Kivu, Tshopo, Lower Uele and Sud Ubangi

64 out of 167 health zones affected (38,3%)

Summary of key points from the last 24 hours

59 new confirmed cases of Ebola Virus Disease (EVD) including 26 community deaths have been reported in 3 provinces: Ituri (29 cases) and North Kivu (26 cases) and Haut Uele (4).

In addition, 11 deaths were recorded in the CTEs (4 in Ituri, 4 in North Kivu and in Tshopo (1) in Haut Uele (2), which brings to 37 the total number of confirmed deaths for the period concerned.

On the other hand, 23 patients (15 in Ituri and 8 in North Kivu) were declared cured after obtaining two negative results in the control test for Ebola virus disease.

As of October 7, 2026, the DRC has recorded **8,787** confirmed cases and **4,242** deaths, representing a case fatality rate of **48.3%.** Ituri remains the epicenter of the epidemic, accounting for **74.5** % of cumulative confirmed cases and **49.2%** of new confirmed cases reported in the last 24 hours.

64 out of 167 health zones (38.3%), spread across seven provinces, have been affected since the start of the epidemic.

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Table 1. Distribution of confirmed cases and deaths by affected province, as of October 7, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	29	6542	3 025	46,2%	28/36 (77,8 %)
North Kivu	26	1811	1 036	57,2%	17/34 (50,0 %)
Haut-Uélé	4	368	156	42,4%	7/13 (53,8 %)
Boss	0	51	19	37,3%	7/23 (30,4 %)
South Kivu	0	3	1	33,3%	1/34 (2,9 %)
Bas-Uélé	0	10	4	40,0%	3/11 (27,3%)
Sud Ubangi	0	2	1	50,0%	1/16 (6,3%)
Total	**59**	**8 787**	**4 242**	**48,3%**	**64/167 (38,3 %)**

Table 2. Distribution of confirmed cases and deaths by province and zone as of October 7, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Cas (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	**6542**	**3025**	**46,2%**	**29**	**14**	**4**	**18**
Give	12	1	8,3%				0
Understand	8	6	75,0%				0
Ariwara	8	2	25,0%				0
Aungba	12	5	41,7%				0
Bamboo	210	47	22,4%				0
have	2	2	100,0%				0
Bunia	1797	596	33,2%	7	1	1	2
Ladies	40	9	22,5%				0
Drodro	15	7	46,7%				0
Fireworks	84	31	36,9%				0
Gethy	9	5	55,6%				0
Flounder	2	0	0,0%				0
Kilo	38	12	31,6%				0
Team	176	107	60,8%				0
Liter	295	148	50,2%				0
Logo	14	7	50,0%	0	0	1	1
Fight	35	9	25,7%				0
distributed	4	1	25,0%				0
Washing	41	17	41,5%				0
Darkness	209	105	50,2%	11	6	1	7
Mangala	366	157	42,9%				0
Mongbwalu	710	337	47,5%				0
Nanny	316	177	56,0%	2	2	0	2

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Nizi	799	289	36,2%	7	3	0	3
Legumes	132	39	29,5%	2	2	0	2
Jungle	14	5	35,7%				0
Rwampara	1118	386	34,5%	0	0	1	1
Tchomia	76	25	32,9%				0
A valves	THAT	493	THAT				0
North Kivu	**1811**	**1036**	**57,2%**	**26**	**11**	**4**	**15**
Me	420	278	66,2%	11	8	2	10
Farm	23	7	30,4%				0
Butembo	289	182	63,0%	3	1	1	2
Ten	1	0	0,0%				0
Kalunguta	63	22	34,9%				0
Cut	613	374	61,0%	5	0	1	1
Source	3	2	66,7%				0
Kyondo	37	20	54,1%				0
Lubero	23	11	47,8%				0
Be anxious.	52	20	38,5%	1	0	0	0
Alimbong	5	3	60,0%				0
Manguredjipa	13	5	38,5%				0
Masereka	36	11	30,6%	2	0	0	0
The museums	140	69	49,3%				0
Mutwanga	20	4	20,0%				0
Oicha	36	17	47,2%	2	1	0	1
Vuh	37	11	29,7%	2	1	0	1
Upper Uele	**368**	**156**	**42,4%**	**4**	**1**	**2**	**3**
Government Mangbetu	44	19	43,2%	1	0	0	0
Dumb	1	1	100,0%				0
Gombari	3	1	33,3%				0
Mathematics	102	44	43,1%				0
Lining	95	46	48,4%	3	1	2	3
Beans	4	2	50,0%				0
Ordinary	119	43	36,1%				0
Boss	**51**	**19**	**37,3%**	0	0	1	1
Bafwasende	7	1	14,3%				0
Kabondo	5	1	20,0%				0
Lubunga	1	0	0,0%				0
Maquis - Kisangani	27	11	40,7%				0
Mangobo	8	5	62,5%			1	1
Boss	2	0	0,0%				0
Wanie - Arugula	1	1	100,0%				0
South Kivu	**3**	**1**	**33,3%**	0	0	0	0
Miti -Murhesa	3	1	33,3%				0
Bas Uélé	**10**	**4**	**40,0%**	0	0	0	0

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Blind	1	1	100,0%				0
Viadana	6	1	16,7%				0
Walk	3	2	66,7%				0
Sud Ubangi	**2**	**1**	**50,0%**	0	0	0	0
Hair	2	1	50,0%				0
Total	**8787**	**4242**	**48,3%**	**59**	**26**	**11**	**37**

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA : Not Applicable.

Table 3. Status of alerts notified by province for October 7, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	49	4	53	23	4	26	0	27	9
Bas Uélé	171	1	172	17	1	154	0	18	0
Ituri	1 143	71	1214	96	71	1 047	0	167	110
North Kivu	1 529	63	1592	222	63	1 307	0	285	148
Sud Ubangi	31	1	32	1	1	30	0	2	0
Boss	134	0	134	7	0	127	0	7	0
Total General	**3 057**	**140**	**3 197**	**366**	**140**	**2 691**	**0**	**506**	**267**

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

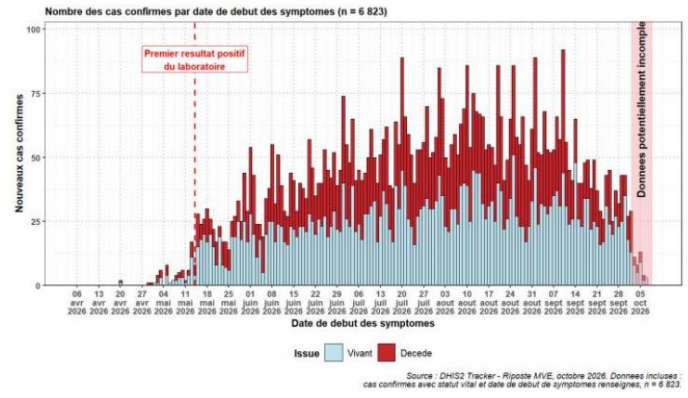
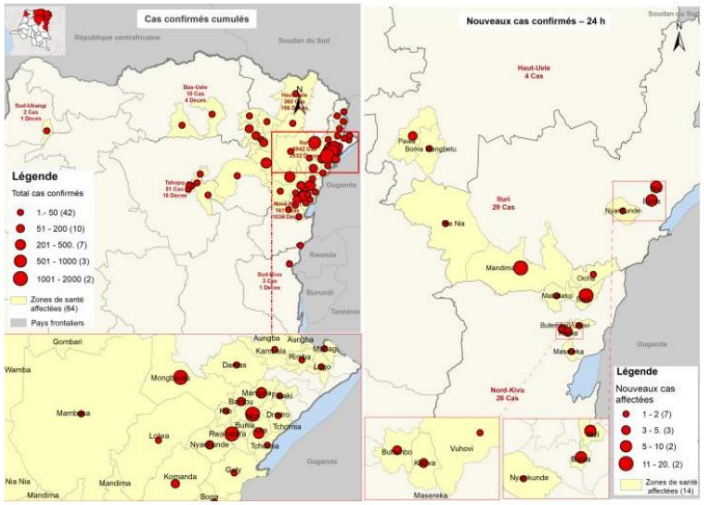


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

• Support for response activities in the affected provinces through the technical pillars.

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2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

On October 7, 2026, **3,197** alerts were reported in the six provinces that had submitted alerts, and all were verified **(100.0%) : 506 (15.8%)** were validated as suspected cases and **2,691 (84.2%)** were invalidated. All 506 validated suspected cases were investigated (100%). Of these, **267 (52.8%)** were transferred to COVID-19 treatment centers (Table 3).

Of the 30,891 contacts currently being monitored, 24,717 were seen in the last 24 hours, representing a follow-up rate of 80.0%, down from the previous day (82.0%) and below the required threshold of 95.0%. By province, this rate was 93.2% (41/44) in Sud-Ubangi, 91.2% (124/136) in Bas-Uélé, 89.2% (222/249) in Tshopo, 89.0% (1,708/1,919) in Haut-Uélé, 83.0% (11,141/13,427) in Ituri, and 76.0% (11,481/15,116) in Nord-Kivu. No province reached the 95% threshold, and Nord-Kivu remains the province with the lowest rate. The health zones of Mutwanga (North Kivu, 956 contacts) and Makiso-Kisangani (Tshopo) have not submitted their follow-up report.

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

Seven (7) alerts were notified to the PoE/PoC listed in Table 4 on October 7: three (3) in Ituri, three (3) in North Kivu, and one (1) in Tshopo; all 5 confirmed suspected cases were investigated within 24 hours. In Ituri, the two alerts from the Mudzi Bala PoC (Bunia Health Zone) were transferred to the CTE CME, and the alert from the Biakato Mai PoC (Mandima Health Zone) was collected after its transfer was refused; the alert from September 28 in Mudzi Bala proved positive. In North Kivu, the alert from the Maboya PoC (Kalunguta Health Zone) was validated and referred to the Tinge Hospital, the alert from the PK5 PoC (Beni Health Zone) was invalidated, and a lifeless body intercepted at the Kasitu PoC (Beni Health Zone) was transported to the morgue for swab

Table 4. Status of key indicators for PoE/PoC by province, as of October 7, 2026

Indicators	Ituri	North Kivu	Boss	Global
Report completeness	93,3% (56/60)	95,2% (20/21) 100.0% (18/18)		94,9% (94/99)
% of functional strategic PoE/PoC	100%	100%	100%	100%
Number of people who have passed PoE/PoC	138 855	94 168	18 813	251 836
% of travelers screened	95,0%	98,1%	98,6%	96,4%
% of travelers who washed their hands	97,5%	98,1%	98,9%	97,8%
% of travelers made aware	99,6%	98,1%	100,0%	99,1%
Number of alerts notified	3	3	1	7
Number of validated live alerts (suspected cases)	3	1	1	5
Number of bodies recovered today	0	1	0	1
% of suspected cases transferred to CTE/CT	66,7%	100,0%	100,0%	80,0%
% of lifeless bodies swabed/secured	THAT	100,0%	THAT	100,0%
% of suspected cases investigated within 4 h	100,0%	100,0%	100,0%	100,0%

2.2.2. Challenges

- **In North Kivu,** the Butuhe Point of Control (PoC) has been out of operation since the security incident of September 26. Report completion remains at 95.2% (20/21), and 1,821 out of 94,168 travelers (1.9%) refused screening and handwashing. In Tshopo, 1.4% of travelers (272/18,813) refused screening and 1.1% (205/18,813) refused handwashing, amid rumors that "the water contains the virus" or "the disease does not exist." In South Kivu, report completion is only 55.6% (5/9). In Bas-Uélé, only 19 out of 41 PoCs are operational.

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notifications are functional (46.3%), no PoE is reinforced (0/11), report completeness is only 83.3% (15/18) and PNHF services are not operational.

2.3. Laboratory

2.3.1. Main actions

- **Ituri: 29 new positive results** (15 living and 14 deceased) out of 175 samples received and analyzed **(positivity: 16.6%).**
- **North Kivu: 26 new positive results** (15 living and 11 dead) out of 180 samples received and analyzed **(positivity: 14.4%).**
- **Haut-Uélé: 4 new positive results** (3 living and 1 death) **out of 24 samples received and analyzed.**
- **Tshopo: 10 new samples (10 blood samples) received and analyzed, all returned negatives**
- **Bas Uélé: 4 samples** (4 blood samples) received and analyzed at the Buta laboratory, **all are negative income.**

2.4. Infection Prevention and Control (IPC/EDS)

2.4.1. Main actions

- **Ituri:** 31 out of 34 rings were opened (91.2%), 56 out of 57 households were decontaminated (98.2%), and 6 out of 7 health facilities were decontaminated (85.7%); no households, social economy enterprises, or schools received kits. Of 106 blood and sanitation alerts (including 8 postponements from the previous day), 78 blood and sanitation tests were carried out (73.6%) and 74 bodies were swabbed, with 22 failures (20.8%).
- **North Kivu:** 25 rings were opened out of 38 (65.8%), with a significant gap in Katwa and Beni, 42 ESS decontaminated out of 46 (91.3%) and 7 households out of 36 (19.4%), none of the decontaminated households having been provided with kits; 17 PCI assessments (score card) were carried out in 17 ESS with an average score of 31.6% and the training of 3 ECUMR teams on PCI/WASH/EDS began in the Beni health zone with the support of WHO.
- **Haut-Uélé:** 3 out of 3 health facilities (100%) have been decontaminated, including the CECCA 16 Hospital and the Maman Kahenga Health Center in Wamba after a suspected case, and the Pawa Ebola Treatment Center (ETC) after a COVID-19 safety assessment (DSA); the laboratory at the Isiro General Referral Hospital (HGR) underwent bio-cleaning, 2 health facilities were assessed (scores of 41.9% in Isiro and 24.0% in Boma Mangbetu), and 29 service providers were briefed. Construction work on the latrines at the Wamba ETC has begun with WHO support. Of 4 DSA alerts, 2 DSAs were carried out (50.0%), 2 bodies were swabed, and one body intercepted at the PK10 Point of Control (Isiro Health Zone) was secured.
- **Tshopo.** 42 out of 42 ESS (100%) and 31 out of 34 households (91.2%) that hosted a confirmed case were decontaminated within 48 hours, and 53 out of 103 ESS (51.5%) received PCI kits.
  As for the EDS, out of 8 bodies expected (7 postponements and 1 alert from the CTE), 3 were swabed (37.5%) and 2 EDS were carried out (CTE and HGR of Makiso-Kisangani).
- **Bas-Uélé:** 12 out of 12 ESS (100%) were followed in Viadana (3), Buta (3) and Ganga (6), with briefing of 36 out of 38 providers (94.7%) and 6 out of 6 hygienists (100%); training of 60 providers and 35 hygienists on IPC continued in Viadana (J2) with the support of FHI 360.

2.4.2. Challenges

- **In Tshopo,** infection prevention and control (IPC) supplies and trained providers remain insufficient; only 2 out of 103 health facilities (1.9%) exceed an IPC score of 80%, the incinerator at the Makiso-Kisangani General Referral Hospital is still out of service, and the Rekapi I and II morgues lack sampling kits and personal protective equipment (PPE). In Bas-Uélé, the Buta General Referral Hospital and the Bomea and Bamaria health centers lack a functional waste area and hygienic latrines, triage for monitored health facilities is not operational due to a lack of dedicated staff, garbage and waste management services are insufficient, and the reporting of community deaths remains low.

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2.4.3. Holistic care

2.4.4. Main actions

- **In Ituri,** 45 new admissions were recorded, compared to 76 discharges, including 15 recoveries. At the end of the day, 360 patients remained in isolation, representing a bed occupancy rate of 35.5% (26% for confirmed cases and 43% for suspected cases), including 31 critically ill patients. 343 out of 355 patients (96.6%) received a hot meal, all children under the LNPE/SLM program received care (12/12 at the CTE, 6/6 at the nursery, and 49/49 in the community), and food and nutritional support began at the Lolwa Treatment Center. The province's total number of recoveries has reached 1,836.

- **In North Kivu,** 142 new admissions were recorded compared to 116 discharges, including 8 recoveries. In total, 519 patients remained in isolation at the end of the day, representing an overall bed occupancy rate of 84.2% (315/374), 84.2% for confirmed beds (169/209) and 84.2% for suspected beds (146/165); 156 patients (33.1%) are being treated in ESS facilities that are not CTE/CT facilities.

- **In Haut-Uélé,** 9 new admissions of suspected cases were recorded, compared to 2 discharges (1 non-case and 1 confirmed death at the Pawa CTE), with no recoveries. A total of 76 patients remained in isolation at the end of the day, including 5 in severe or critical condition, representing an occupancy rate of 49.7% (54/153), with 29.6% for confirmed beds and 41.7% for suspected beds; 57 patients, 68 caregivers, and 70 people with disabilities received hot meals.

- **In Tshopo,** one new admission of a suspected case, a nurse who died less than 3 hours after arrival, was recorded, compared to 7 discharges (6 non-cases and 1 death). A total of 15 patients (6 confirmed and 4 suspected) remained in isolation at the end of the day, representing an occupancy rate of 27.8% (10/36); since the beginning, 393 people have received the experimental treatment, including 37 confirmed cases, and 357 have benefited from post-exposure prophylaxis.

- **In Bas-Uélé,** no new confirmed cases were admitted; 10 suspected cases are isolated in non-standard transit centers in Ganga, Poko, Bili, Dakwa, Likati and Limba (ZS of Viadana), and 3 suspects from Buta were declared non-cases after analysis.

2.4.5. Challenges

- **In Ituri,** the low capacity of some CTE/CTs (Mambasa, Jina, Sota Mangala, Mandima and Lolwa), the care of 31 critical patients and the lack of food for 12 patients in Mandima remain worrying.

- **In North Kivu,** the saturation of suspected cases (146/165; 88.5%) and the management of 156 patients (33.1%) outside of CTE/CT remain a concern.

- **In Haut-Uélé,** inconsistencies in care data and the absence of a hearse at CTE in Boma Mangbetu still presents challenges to be solved.

- **In Tshopo,** the demotivation of 12 out of 29 ambulance workers (41.4%) undermines the transfer of patients.

- **In Bas-Uélé,** there is a lack of a standardized CTE (Emergency Medical Treatment Center), which is still under construction, and of an ambulance for transfers. Managing patients remains a major operational challenge.

2.5. Vaccination

2.5.1. Main actions

- **In Bas-Uélé,** the vaccination of PPL and TPL continued in the Poko health zone, with the pre-registration of religious leaders, RECO, traditional healers, motorcyclists and hunters, bringing the provincial total to 1,412 people vaccinated out of a target of 13,202 (10.7%), mainly in Buta (620; 21.6%), in Ganga (273; 20.5%) and in Poko (150; 11.0%).

- **In Ituri,** the Partners, Oracle and EBO-PEP clinical trials continued with the support of INRB, Kivu and ALIMA, and a research protocol on the impact of the epidemic on the use of sexual and reproductive health services was received for analysis.

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2.5.2. Challenges

In Ituri, vaccination of PPL and the study of cross-immunity with the BDBV strain remain suspended pending the supply of Ervebo vaccine by the national PEV.

In Bas-Uélé, the implementation of the UCCs is not effective and the validated micro-plan as well as the vaccination funds are still not available.

2.6. Continuity of Care

2.6.1. Main actions

- **In Tshopo,** the evaluation of the Tshopo 2 Health Center and the Salama Health Center revealed the absence of an effective triage area and waste management system (no incinerator, no garbage pit, no placenta pit), the non-use of the partogram during deliveries, low staff vaccination at the Salama Health Center (2 out of 15 staff; 13.3%) and 5 missed ANC appointments out of 16 expected (31.3%); the October PCSSE work plan and the recovery plan for the City Health Center were finalized, and the monitoring of PPL at the CTM and CTE as well as that of recovered patients with comorbidities at the Makiso-Kisangani General Referral Hospital continued.

- **In Haut-Uélé,** a rapid survey on the reasons for the low use of healthcare services has started in the Kolu health district (Isiro health zone), with 28 households surveyed out of approximately one hundred targeted.

2.7. Risk Communication and Community Engagement (RCCE)

2.7.1. Main actions

- **In Ituri,** 30,840 home visits reached 51,032 people; 741 educational talks reached 20,278 people, 323 mass awareness campaigns reached 4,780 people, 3 community debates reached 544 people, and 12 advocacy sessions were held with 24 authorities and leaders; 34 public service announcements were broadcast on 36 radio stations, and 11 programs were organized. A total of 346 community alerts, including 96 deaths, were reported; 123 out of 151 feedback requests (81.5%) and 7 out of 8 incidents (87.5%) were resolved within 48 hours, and a guided tour of the Rwampara Ebola Treatment Center (ETC) was organized for approximately ten community health workers (CHWs) from the Dele Health Association.

- **In Bas-Uélé,** 123 home visits reached 482 people in Ganga and Viadana, 3 mass awareness sessions reached 900 people and a radio broadcast on RVA/Aketi reached 13,859 listeners; 41 service providers in the Viadana health zone and 8 service providers at the PK65 checkpoint were briefed, and the 2 community alerts, 5 feedbacks and the reported incident were all resolved (100%).

- **In North Kivu,** 7,790 home visits reached 23,427 people, 188 educational talks reached 5,902 people, and 25 mass communication sessions reached 5,051 people; 269 community alerts out of 650 expected (41.4%) were reported, 26 feedback reports out of 34 (76.5%), and 2 out of 5 incidents (40.0%) were resolved within 48 hours; and 1,914 people out of 4,750 expected (40.3%) participated in community actions. The discharge of 8 recovered patients from the Ebola Treatment Center (ETC) in Beni was covered by 16 media outlets, and 60 school principals in Beni and 111 teachers in Manguredjipa were mobilized for prevention in schools.

- **In Tshopo,** 5 mass awareness sessions reached 15,506 people

  Bafwasende, Tshopo, Makiso-Kisangani and Lubunga, 217 home visits to 1,519 people and 3 educational talks to 214 people; all 63 community alerts notified were resolved, 2 out of 2 feedback requests and 2 out of 2 refusals were lifted, and 128 leaflets and 174 posters were distributed.

- **In Haut-Uélé,** 24 educational talks, 4 mass awareness sessions and 71 home visits reached 1,412 people in Isiro and Wamba; 52 community alerts, including 4 deaths, were notified in 6 health zones and the 7 feedback received were resolved (100%), most of which concerned communication around CTE patients, and 30 media actors in the province were trained in CREC in emergency situations.

2.8. Mental Health and Psychosocial Support (MHPSS)

2.8.1. Main actions

- **In North Kivu,** 20 out of 23 confirmed cases (87.0%), 46 confirmed cases under follow-up, 93 new suspected cases, and 676 suspected cases under follow-up benefited from SMSPS interventions; the teams supported 70 households with public health activities and 23 households with separated children and

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orphans out of 25 (92.0%), supported 60 PPL, followed up 11 cured out of 11 in community (100%), 6 out of 6 children in nursery and 31 out of 35 children in community (88.6%), carried out psychoeducation of 364 people (127 men, 196 women and 41 children) with the support of UNICEF and announced 125 results, including 23 positive ones; 7 out of 9 health zones that notified are covered (77.8%).

- **In Ituri,** 24 out of 34 confirmed cases (70.6%), 91 out of 132 confirmed cases under follow-up (68.9%), 30 out of 45 new suspected cases (66.7%), and 112 out of 158 suspected cases under follow-up (70.9%) benefited from SMSPS interventions. The teams supported 270 caregivers and 98 people living with HIV, monitored 18 out of 20 recoveries (90.0%) in the community and 15 out of 15 children in daycare, provided support to 24 out of 27 households with separated and orphaned children (88.9%), reintegrated 11 out of 15 recoveries (73.3%), and reported 34 results, 24 of which were positive.

- **In Tshopo,** in the absence of a new confirmed case, the 6 confirmed cases under follow-up, the new suspect and the 13 suspects under follow-up have been taken care of (100%); the teams supported 13 PPLs and 3 households, lifted 2 out of 2 resistances around the households of contacts linked to DHKis and carried out psychoeducation of 121 people (52 contacts in Makiso-Kisangani, 17 in Mangobo, 3 in Lubunga, 36 accompanying persons at the CTE Météo and 13 PPLs); 5 out of 7 ZSs that notified are covered (71.4%).

- **In Haut-Uélé,** 5 recent confirmed cases, 26 confirmed cases under monitoring, 4 new suspected cases and 20 suspected cases under monitoring benefited from SMSPS interventions in the Ebola Treatment Centers of Isiro, Pawa, Boma Mangbetu and Wamba; the teams supported 49 caregivers, 82 PPLs (including 25 members of the national coordination), 10 children separated in the community and 15 household members, supported 3 EDS and announced 37 results, including 8 positive ones

- **In Bas-Uélé,** 5 PPLs, 6 household and community members and 2 contacts were supported, 1 discharged cured person is being monitored, 1 reluctance was lifted and 3 results were announced (all negative); a joint visit with the CREC was conducted with the husband of the last confirmed case of Limba for his adherence to compassionate treatment.

2.8.2. Challenges

- **In Ituri, only** 14 out of 28 affected health zones are covered (50.0%) and the Mandima health zone has no SMSPS coverage, which requires strengthening the capacities of psychologists and APS/TPS to ensure interventions around cases.

- **In Bas-Uélé,** SMSPS coverage remains embryonic, with activities having only just begun in the Viadana ZS.

- **In Haut-Uélé,** the mobility of community health workers remains difficult in Wamba and Boma Mangbetu, nutritional kits are lacking for weaned infants in Wamba and Isiro, the community health worker lacks computer equipment, and data collection remains weak in Boma Mangbetu and Pawa

- **In Tshopo,** there is a need to train the APS of the ZS of Wanie-Rukula and Bafwasende and a shortage of communication units and motorcycles for the movement of the pillar.

2.9. Logistics

2.9.1. Main actions

- **In Bas-Uélé,** receipt of IPC inputs offered by FHI 360, deployment of surveillance teams in Ganga and Viadana for contact tracing, verification of alerts and active search, and continuation of the rehabilitation of the meeting room (fixing of windows) and the Hygiene and Education offices of the DPS with the support of the WHO, as well as the construction of the CTE of the HGR of Buta, whose perimeter walls are completed; the DPS no longer has any stock of diesel.

- **In Tshopo,** monitoring of the construction of the Bafwasende CTE, the extension of the Meteorological CTE and the isolation of Konga-Konga, whose work will resume after the completion of the drainage ditches, unpacking of the PHATKIN medicines in Bafwasende, start of inventories at the depot and support for the mobility of the PCI, PEC, Communication and PoE/PoC pillars as well as the return to Bafwasende of high risk contacts; the mobility need of the ZS remains estimated at 154 motorcycles.

- **In Haut-Uélé,** 6 ambulance trips transported 6 patients to Pawa (2), Wamba (2), Isiro (1) and The Boma Mangbetu (1)

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2.10. Security

2.11. Main actions

- **In Tshopo,** community resistance to investigations, sampling, isolation, and transfer to the Ebola Treatment Center (ETC) persists, and physical attacks against rapid intervention teams continue to be reported.

- **In Bas-Uélé,** the security situation remained calm at the intervention sites; response activities were secured, and discussions took place with security service officials, the ANR (National Intelligence Agency), and the DGM (General Directorate of Migration) regarding support for the search for potential contacts hidden within the community.

- **In North Kivu,** the decontamination team was attacked by the population at the Boikeni Health Center before being freed by the security services, who arrested the perpetrators; the EDS and PCI teams were escorted for burials at the cemeteries of Paida (Beni Health Zone) and Lusovovo (Katwa Health Zone), the swab of a body at the Kivika Hospital (Katwa Health Zone) and the decontamination of the La Modestie and Baraka Medical Centers as well as a household in the Kasanga Health Area (Beni Health Zone).

2.12. PSEA (Prevention of Sexual Exploitation and Abuse)

2.12.1. Main actions

- **In Ituri,** the training of trainers on the PSEAH (Protection of Children and Adolescents with Disabilities) program began in the Bunia health zone with 27 participants (19 women and 8 men). Training sessions brought together 61 teachers and members of parent committees in Komanda (23 women and 38 men), as well as 22 members of COPA (Community Action Plans), teachers, community leaders, and women's associations in Tchomia (8 women and 14 men). Awareness campaigns reached 3,502 students and staff from five schools in Aru, 22 community and traditional leaders in Lita, and 8 painters in Bunia. Seventy-two staff members were briefed and signed the code of conduct. The posting of messages and the distribution of leaflets continued in Bunia, Aru, Mongbwalu, Mambasa, and Fataki.

- **In North Kivu,** briefings on the PSEAH were carried out in the health zones of Beni, Butembo and Katwa, and 1,097 people were sensitized on the PSEA and reporting channels (495555 and 151) in Beni (168), Butembo (411), Katwa (465), Mutwanga (21) and Biena (32), with the distribution of 187 leaflets and 10 posters.

2.12.2. Challenges

At the national level, only 2 out of 7 DPS (28.6%) reported their PSEA activities; in Ituri, only 13 out of 36 ZS (36.1%) know the reporting mechanism, activities do not yet cover all affected ZS and only 3 partners (WHO, IMC and Save the Children) report their PSEA activities.

MESSAGES CLES

ÿ In the provinces of Ituri, North Kivu and Haut-Uélé, it is observed
active community transmission while sporadic notification is observed in Tshopo province.

2. In the last 24 hours.

ÿ Over the past 24 hours, all suspected cases (100.0%) have been investigated; performance is consistent with data reported by the provinces. However, contact tracing remains inconsistent (80.0% vs. 82.0%) and warrants close attention to prevent uncontrolled chains of transmission.

Maintain a level of vigilance regarding epidemiological surveillance aspects, consolidate IPC and CREC actions at the community level while providing quality care in dedicated care structures with adequate reception capacities.

TIMELINE OF KEY EVENTS

ÿ From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.

ÿ Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.

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